

Recommendation

Lateral resection margins for primary cutaneous melanoma should be determined based on the depth of tumor infiltration, as follows:

- **In situ melanoma:** 0.5 cm margin
- **0.1–1 mm melanoma:** 1 cm margin
- **1.1–2 mm melanoma:** 1–2 cm margin
- **>2 mm melanoma:** 2 cm margin

(Recommendation Grade B)

No robust evidence is available regarding optimal resection depth. Therefore, the project team recommends resection down to the fascia, while preserving the fascia when possible.

(Recommendation Grade C)

For cases of Dubreuilh melanoma or lentigo maligna melanoma

The project team supports maintaining the 2005 guidelines for surgical management of in situ lentigo maligna-type melanomas. Data on imiquimod (which lacks marketing authorization for this indication) and radiotherapy are insufficient to establish definitive clinical guidelines for routine use. However, in rare cases of inoperable melanoma, these treatments may be considered on a case-by-case basis, preferably within clinical trials.

Recommendation

- A **1 cm margin** is recommended for Dubreuilh melanomas.

- If a 1 cm margin is not feasible for anatomical or functional reasons, a **0.5 cm margin** may be accepted, provided that strict histological margin control is performed using techniques such as Mohs surgery or equivalent methods with intraoperative margin assessment, or via a two-stage surgical approach. (Expert opinion)
- In very rare cases of inoperable melanoma, **radiotherapy or imiquimod** (off-label use) may be considered following multidisciplinary team (MCM) discussion. (Expert opinion)

Topic 3: Sentinel lymph node biopsy

The available literature primarily focuses on intermediate and final results from the MSLT1 study.

Sentinel lymph node biopsy (SLNB) allows accurate staging and is a strong independent prognostic factor for melanoma. However, it has not been shown to improve overall survival. Therefore, it cannot be considered standard treatment. It may be offered for melanomas >1 mm in thickness, provided the patient is fully informed about the lack of proven therapeutic benefit. SLNB should only be performed in centers with established expertise in the technique.

Recommendation

- **SLNB is an option** for melanomas >1 mm thick. If performed, it must be carried out in a center with adequate expertise.
- **SLNB is not recommended** for melanomas <1 mm thick. (Recommendation Grade A)
- SLNB may also be considered within clinical trials, particularly in the context of adjuvant therapy. (Expert opinion)

Role of lymph node dissection

When sentinel lymph node biopsy is not performed, published data on systematic lymph node dissection have not evolved since the 2005 recommendations, which remain valid.

For cases where SLNB is performed, current evidence does not definitively support whether immediate completion lymph node dissection (CLND) after a positive SLNB improves outcomes. Until results from the MSLT2 study are available, no evidence-based recommendation can be made. In France, the prevailing practice is to perform immediate CLND upon detection of a positive sentinel node, although this approach lacks strong supporting evidence.

Recommendation

- **Systematic lymph node dissection is not recommended** in Stage I–II melanomas when SLNB has not been performed. (Evidence Level A)
- When SLNB is positive, there is no conclusive evidence to favor immediate dissection over delayed dissection at clinical relapse. The project team endorses continuing the current common practice of **immediate lymph node dissection** in such cases. (Expert opinion)

Topic 4: Adjuvant treatments

Medical treatments

Currently, **interferon** is the only medical treatment considered for adjuvant therapy in high-risk melanoma. However, its impact on overall survival remains uncertain, leading to variable international recommendations ranging from "not recommended" to "recommended."

Other agents evaluated—such as gangliosides, **ipilimumab**, and **bevacizumab**—have not demonstrated proven efficacy in improving overall survival and are

not authorized for use in melanoma. Therefore, they cannot be recommended for routine clinical use.